

Name : Ms. TINA PARDESHI	Age : 20 Years
Lab No. : 382967083	Gender : Female
Ref By : SELF.....	Reported : 07/09/2026 07:41:12 AM
Collected : 06/09/2026 04:59:00 PM	Report Status : Final
A/c Status : P	
Collected At : M K PATHOLOGY	Processed At : CPL
VINAYAK COMPLEX, SANJAY JALASAY ROAD, NEAR DR. ASOLIYA CLINIC, PITHAMPUR	Omega Tower Indore
Id :	



MC-6624

Test Report

Test Name	Result	Unit	Biological Reference Interval
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COMPLETE BLOOD COUNT (CBC), WHOLE BLOOD (Electrical Impedence, Photometric)

Hemoglobin	11.70	g/dL	(Spectrophotometry) 12.00 - 15.00
Packed Cell Volume (PCV)	38.00	%	(Analogical Integration) 36.00 - 46.00
RBC Count	4.73	mill/mm3	(Electrical Impedence) 3.80 - 4.80
MCV	80.3	fL	(Calculated) 83.00 - 101.00
MCH	24.7	pg	(Calculated) 27.00 - 32.00
MCHC	30.8	g/dL	(Calculated) 31.50 - 34.50
Red Cell Distribution Width (RDW)	13.9	%	(Calculated) 11.60 - 14.00

Total Leukocyte Count (TLC)	6290	/cmm	(Electrical Impedence & Flow) 4400 - 11000
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Differential Leucocyte Count (DLC)

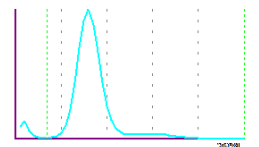
Segmented Neutrophils	44.70	%	(Flow Cytometry) 40.00 - 80.00
Lymphocytes	42.10	%	(Flow Cytometry) 20.00 - 40.00
Monocytes	5.60	%	(Flow Cytometry) 2.00 - 10.00
Eosinophils	7.00	%	(Flow Cytometry) 1.00 - 6.00
Basophils	0.60	%	(Flow Cytometry) <= 2.00

Absolute Leucocyte Count (Calculated)

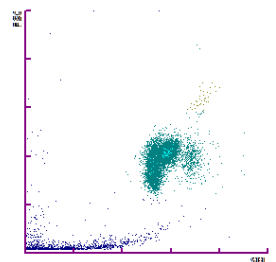
Neutrophils	2812	/cmm	(Calculated) 1800 - 7800
Lymphocytes	2648	/cmm	(Calculated) 1000 - 4800
Monocytes	352	/cmm	(Calculated) 120 - 1200
Eosinophils	440	/cmm	(Calculated) 0 - 450
Basophils	38	/cmm	(Calculated) 0 - 100
Platelet Count, Whole Blood	290	thou/mm3	(Electrical Impedence) 150.00 - 450.00
MPV (Mean Platelet Volume)	9.50	fL	(Calculated) 6.50 - 12.00

Note

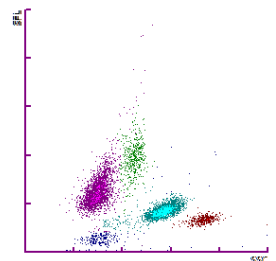
- As per the recommendation of international council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.
- Test conducted on EDTA whole blood.



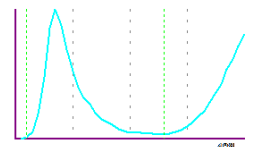
RBC



WNR



WDF



PLT

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Test Name	Results	Units	Bio. Ref. Interval
LIVER & KIDNEY FUNCTION TEST (LFT & KFT), SERUM			
Urea (Urease,UV)	22.40	mg/dL	18 - 45
Urea Nitrogen Blood (25997) (Urease Colorimetric)	10.47	mg/dL	6.00 - 20.00
BUN/Creatinine Ratio_new (Calculated)	15.86		
CREATININE, SERUM (Jaffe Compensated)	0.66	mg/dL	0.6 - 1.4
GFR, Estimated (Calculated)	128.71	ml/min/1.73m	> 59
GFR Category -B023	G1		
URIC ACID, SERUM (Enzymatic Colorimetric)	2.40	mg/dL	2.4 - 5.7
AST (SGOT) (IFCC without P5P)	16.60	U/L	0 - 32
ALT (SGPT) (IFCC without P5P)	8.70	U/L	<33
GGTP (IFCC)	22.00	U/L	0 - 42
Alkaline Phosphatase (ALP) (IFCC)	53.00	U/L	0 - 329
Bilirubin Total (Diazo)	1.03	mg/dL	0 - 1.1
Bilirubin Direct (Diazo)	0.24	mg/dL	0 - 0.2
Bilirubin Indirect (Calculated)	0.79	mg/dL	0.1 - 1.2
Total Protein (BIURET)	7.15	g/dL	6.4 - 8.3
Albumin (BCG)	4.22	g/dL	3.5 - 5.2
Globulin (Calculated)	2.93	g/dL	2.00 - 3.50
A : G Ratio (Calculated)	1.44		0.90 - 2.00
Calcium, Serum (NM-BAPTA)	9.31	mg/dL	8.6 - 10
Phosphorus (Molybdate UV)	3.55	mg/dL	2.6 - 4.5
Sodium (ISE)	140.30	mEq/L	136 - 145
Potassium (ISE)	4.98	mEq/L	3.5 - 5.1
Chloride (Indirect ISE)	101.50	mEq/L	98 - 108



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Test Name	Results	Units	Bio. Ref. Interval
GLUCOSE FASTING (F), PLASMA (Hexokinase)	84.80	mg/dL	70 - 110
HbA1c, WHOLE BLOOD			
HbA1c	5.7	%	Non Diabetic (Normal):- less than 5.7% Prediabetes (at risk):- 5.7% to 6.4% Diabetes:- 6.5% or higher
(HPLC, NGSP certified)			
Estimated average glucose (eAG) (Calculated)	116.89	mg/dL	90 - 150

Interpretation

HbA1c result is suggestive of non diabetic adults (≥18 years)/well controlled Diabetes in a known Diabetic

Interpretation as per American Diabetes Association (ADA) Guidelines

Reference Group	Non Diabetic adults	At risk	Diagnosing	Therapeutic goals
	adults ≥18years	(Prediabetes)	Diabetes	for glycemic control
HbA1c in %	4.0 - 5.6	5.7 - 6.4	≥6.5	<7.0

Note Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels

FACTORS THAT INTERFERE WITH HbA1C MEASUREMENT	FACTORS THAT AFFECT INTERPRETATION OF HbA1C RESULTS
Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements	Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g., recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, & HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c



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Test Name	Results	Units	Bio. Ref. Interval
THYROID PROFILE,TOTAL, SERUM			
T3, TOTAL, SERUM (ECLIA)	1.35	ng/mL	0.80 - 2.00
T4, TOTAL, SERUM (ECLIA)	9.52	µg/dL	4.87 - 12.23
TSH (THYROID STIMULATING HORMONE), SERUM (ECLIA)	1.68	µU/mL	0.27 - 4.2

Note:

1. TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm .The variation is of the order of 50% . hence time of the day has influence on the measured serum TSH concentrations.
2. Alteration in concentration of Thyroid hormone binding protein can profoundly affect Total T3 and/or Total T4 levels especially in pregnancy and in patients on steroid therapy.
3. Unbound fraction (Free,T4 /Free,T3) of thyroid hormone is biologically active form and correlate more closely with clinical status of the patient than total T4/T3 concentration
4. Values <0.03 uIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals

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Test Name	Results	Units	Bio. Ref. Interval
LIPID PROFILE, SCREEN, SERUM			
CHOLESTEROL TOTAL, SERUM	204.60	mg/dL	Optimal: <200 Borderline high: 200 - 239 High: >=240
(CHOD-PAP)			
TRIGLYCERIDES, SERUM	48.40	mg/dL	< 150
(CHOD - POD)			
HDL Cholesterol	67.10	mg/dL	40 - 60
(HomoGenous Enzymatic Colorimetric)			
LDL Cholesterol, Direct	140.70	mg/dL	<100
(Enzymatic)			
VLDL Cholesterol	9.68	mg/dL	<30.00
(Calculated)			
Non-HDL Cholesterol	137.50	mg/dL	<130.00
(Calculated)			

Note

- Measurements in the same patient can show physiological & analytical variations. Three Serial samples 1 week apart are recommended for Total Cholesterol, Triglycerides, HDL & LDL Cholesterol.
- Friedwald equation to calculate LDL cholesterol is most accurate when Triglyceride level is < 400 mg/dL. Measurement of Direct LDL cholesterol is recommended when Triglyceride level is > 400 mg/dL.
- Lipid Association of India (LIA) recommends screening of all adults above the age of 20 years for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors especially lipid profile. This should be done earlier if there is family history of premature heart disease, dyslipidemia, obesity or other risk factors.
- Indians tend to have higher triglyceride levels & Lower HDL cholesterol combined with small dense LDL particles, a pattern known as atherogenic dyslipidemia.
- Non HDL Cholesterol comprises the cholesterol carried by all atherogenic particles, including LDL, IDL, VLDL & VLDL remnants, Chylomicron remnants & Lp(a).
- LIA recommends LDL cholesterol as primary target and Non HDL cholesterol as co-primary treatment target.
- Apolipoprotein B is an, secondary lipid target for treatment once LDL & Non HDL goals have been achieved.
- Additional testing for Apolipoprotein B, hsCRP, LP(a) & LP-PLA2 should be considered among patients with moderate risk of ASCVD for risk refinement.

Treatment Goals as per Lipid Association of India 2020

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RISK CATEGORY	TREATMENT GOAL		CONSIDER THERAPY	
	LDL CHOLESTEROL (LDL-C) (mg/dL)	NON HDL CHOLESTEROL (NON HDL-C) (mg/dL)	LDL CHOLESTEROL (LDL-C) (mg/dL)	NON HDL CHOLESTEROL (NON HDL-C) (mg/dL)
Extreme	< 50	< 80	≥50	≥80
Risk Group	(Optional goal ≤30)	(Optional goal ≤60)		
Category A				
Extreme	≤30	≤60	>30	>60
Risk Group				
Category B				
Very High	<50	<80	≥50	≥80
High	<70	<100	≥70	≥100
Moderate	<100	<130	≥100	≥130
Low	<100	<130	≥130*	≥160*

*In low risk patient, consider therapy after an initial non-pharmacological intervention for at least 3 months

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Test Name	Results	Units	Bio. Ref. Interval
VITAMIN D, 25 - HYDROXY, SERUM	7.73	ng/mL	Deficiency: Below 10 Insufficiency: 10 to 30 Sufficiency: 30 to 100 Toxicity: Above 100
(ECLIA)			

Interpretation

LEVEL	REFERENCE RANGE IN nmol/L	COMMENTS
Deficient	< 50	High risk for developing bone disease
Insufficient	50-74	Vitamin D concentration which normalizes Parathyroid hormone concentration
Sufficient	75-250	Optimal concentration for maximal health benefit
Potential intoxication	>250	High risk for toxic effects

Note

- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D.
- 25 (OH)D is influenced by sunlight, latitude, skin pigmentation, sunscreen use and hepatic function.
- Optimal calcium absorption requires vitamin D 25 (OH) levels exceeding 75 nmol/L
- It shows seasonal variation, with values being 40-50% lower in winter than in summer.
- Levels vary with age and are increased in pregnancy.
- A new test Vitamin D, Ultrasensitive by LC-MS/MS is also available

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Test Name	Results	Units	Bio. Ref. Interval
VITAMIN B12, SERUM (ECLIA)	338.0	pg/mL	180 - 914

DR. SHWETA RAGHUVANSHI
MBBS, DCP, DNB PATHOLOGY

----- End Of Report -----



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